CMS-0042-NC
Request for Information:
Health Technology Ecosystem

Introduction to Rhapsody Health's RFI Response

Rhapsody Health appreciates the opportunity to respond to the Centers for Medicare & Medicaid Services (CMS) and the Office of the National Coordinator for Health Information Technology (ONC) on shaping the future of the national health technology ecosystem. As a global leader in healthcare data interoperability and digital enablement, we are uniquely positioned to support CMS's strategic vision of a connected, equitable, and patient-empowered healthcare system.

With over 1,700 healthcare organizations relying on our platform across 17 countries—including health systems, payers, government agencies, public health networks, and digital health innovators—Rhapsody offers deep technical and operational expertise in the movement, normalization, governance, and secure exchange of healthcare data. Our technologies serve as the connective tissue behind thousands of clinical and administrative workflows every day, facilitating critical functions such as patient identity resolution, claims exchange, remote monitoring integration, and real-time clinical decision support.

We bring to this RFI a track record of:

- **Proven performance in national and regional interoperability initiatives,** including HIEs, TEFCA-aligned networks, and FHIR API implementations.
- **Leadership in semantic normalization and terminology services,** ensuring data retains its meaning and utility across systems.
- **Advanced EMPI capabilities,** supporting accurate patient and provider identity resolution—essential for longitudinal records and digital identity frameworks.
- **Support for modern interoperability standards,** including HL7 v2, FHIR R4, CDA, and X12, alongside emerging paradigms like Bulk FHIR, SMART on FHIR, and CDS Hooks.

Beyond compliance, Rhapsody is committed to enabling a new generation of digital health applications—those that empower patients, ease provider burden, enhance care navigation, and drive value-based outcomes. Our perspective is informed not only by our technology, but by our role as an ecosystem integrator: bridging gaps between stakeholders, systems, and data silos to realize CMS's vision of a learning health system.

We are excited to share insights, capabilities, and ideas across all relevant domains of this RFI and stand ready to support CMS in building a resilient, interoperable, and patient-centered health technology future.

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Stakeholder Category – Patients and Caregivers

PC-1. Desired Health Management and Care Navigation Apps

Rhapsody Health enables digital health tools to deliver on the promise of patient empowerment by integrating disparate data sources into a unified, longitudinal patient view. Our technology powers applications that support chronic disease management, wellness tracking, care coordination, and shared decision-making.

- Seamless integration of EHR, claims, lab, pharmacy, and SDOH data into downstream applications.
- Support for SMART on FHIR to connect with patient-authorized apps.
- Support for mobile and web-based health navigation apps for scheduling, reminders, and personalized insights.

Idea/Recommendation: CMS could promote a new certification for "Patient-Ready Apps" that demonstrates usability, safety, and API interoperability with Medicare systems. CMS might also support a centralized app registry where beneficiaries can browse and select trusted digital tools validated for Medicare use.

PC-2. Unified Access to Health Information

Rhapsody's platform allows organizations to aggregate health information across systems into a single view. Our EMPI ensures accurate record linking across sources, which is essential to creating a seamless patient portal experience.

- Aggregate data from across multiple EHRs, HIEs, payers, and government registries.
- Match patient identities across systems with high precision using referential, probabilistic, and rules-based methods.
- Normalize data for presentation in consumer-facing applications using standardized terminologies.

Idea/Recommendation: CMS could sponsor an open-source patient data dashboard prototype that app developers can build upon, creating a baseline user experience that prioritizes inclusivity, clarity, and integration. TEFCA participation could be extended to include patient-facing apps as recognized endpoints, further supporting unified data access.

PC-3. Awareness of Relevant Digital Tools

Through our partnerships with digital health innovators and providers, Rhapsody has enabled applications that:

- Support personalized medication tracking
- Offer appointment coordination and real-time provider lookups
- Provide caregiver-controlled access to dependent records

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Our interoperability capabilities have made these apps viable by enabling secure, scalable access to FHIR and non-FHIR sources.

Idea/Recommendation: CMS could host "showcase challenges" that feature top-performing digital health apps used by Medicare beneficiaries, encouraging market innovation while surfacing best practices in usability and integration.

PC-4. Missing Features and CMS's Potential Role

Most current apps still require patients to manually authenticate and connect to each health system, and often lack real-time updates or full claims data access. Rhapsody reduces these barriers by connecting fragmented data sources in real-time and wrapping legacy data in modern APIs.

Idea/Recommendation: CMS could pilot a federated credential system that allows one-time patient login to multiple systems. CMS could also offer "starter APIs" for Medicare-participating providers who lack the infrastructure to expose their data directly.

PC-5. Driving Adoption of Digital Health Tools

Rhapsody enables the underlying connectivity that powers trusted, secure digital experiences for patients. Our role often includes ensuring that data integrity, access rights, and privacy controls are honored across connected systems.

Idea/Recommendation: CMS could consider a voluntary digital product endorsement framework, allowing high-performing apps to be listed in a CMS "beneficiary app marketplace." Apps that meet rigorous criteria for usability, accessibility, and data privacy could earn badges similar to ONC Health IT Certification.

PC-6. Accessibility and Ease of Use for Beneficiaries

Rhapsody's integration capabilities allow developers to present complex medical information in user-friendly formats, including:

- Simplified language translations of clinical terminology
- Voice interfaces via connected platforms (e.g., Alexa, Siri)
- Configurable caregiver access that honors legal guardianship and consent models

Idea/Recommendation: CMS could develop accessibility design standards tailored to seniors and individuals with disabilities, and require adherence for any app seeking to access Medicare APIs. Incorporating community feedback from senior centers and disability advocacy groups would help shape these standards.

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PC-7. Real-World Data Collection on Product Impact

Rhapsody helps partners collect data on digital health tool usage, engagement, and outcomes by integrating usage data back into clinical and claims systems for analysis.

Idea/Recommendation: CMS could standardize the reporting of app usage analytics (e.g., time spent, features used, care impact) through anonymized, opt-in metrics collected via SMART on FHIR launch parameters. This data could feed a real-world evidence platform for evaluating digital product efficacy.

PC-8. Availability and Accessibility of Health Data

Rhapsody supports real-time data access for:

- Demographics, medications, allergies, immunizations
- Claims data**,** including Part A, B, and D through Blue Button integration
- Lab and imaging results, when provided in HL7, CDA, or FHIR

Our normalization tools make this data actionable in patient apps.

Idea/Recommendation: CMS could encourage participation in cross-sector partnerships that integrate social services data, transportation, and community resource directories to enhance the value of clinical apps beyond the medical domain.

PC-9. Enhancing Blue Button 2.0

Rhapsody integrates with Blue Button 2.0 today to enable Medicare data in consumer health apps. The platform could be further improved by adding:

- Provider encounter notes
- Real-time prior authorization status
- Care team directory and contact info

Idea/Recommendation: CMS could adopt a modular FHIR API framework for Blue Button 3.0, offering opt-in tiers of data sharing that balance patient utility and provider system capacity. A developer sandbox and test data generator would also foster ecosystem innovation._

PC-10 & PC-11. TEFCA and HIEs' Role in Access

Rhapsody is actively supporting clients preparing for TEFCA QHIN connectivity, and is already integrated with leading HIEs across the U.S.. These frameworks are essential for enabling cross-system patient data retrieval**.**

Idea/Recommendation: TEFCA should extend implementation guides and testing tools to include patient-facing apps, HIEs, and integration vendors like Rhapsody. CMS might consider funding pilots that connect rural HIEs to national networks through intermediaries.

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PC-12. Operational Use Cases that Could Transform Access

Rhapsody supports:

- Binding cost estimates via price transparency integrations
- Provider search with real-time schedule availability
- Appointment booking with FHIR Appointment resources
- Third-party app integration with scheduling and intake systems

Idea/Recommendation: CMS could standardize a "Provider Interact API Suite" offering low-barrier access to scheduling, price estimates, and care quality data via one developer-friendly framework.

PC-13. Encouraging Information Blocking Complaints

Rhapsody empowers patients and developers to recognize when data access is being impeded by surfacing logging and access errors through API instrumentation.

Idea/Recommendation: CMS could launch a one-click "Report Blocking" widget that app developers can embed in their apps to report non-compliant behavior. Public reporting of compliance scores could drive behavior change.

PC-14. Digital Identity Credential Challenges

Rhapsody supports integration with digital identity providers like Login.gov, ID.me, CLEAR, enabling credential mapping to patient records via EMPI.

Idea/Recommendation: CMS could pilot a unified Medicare digital ID that supports both web and in-person identity proofing. Partnering with libraries, pharmacies, and community centers could improve credential access among underserved populations.

Stakeholder Category – Provider

PR-1. Encouraging Provider Adoption of Digital Health Products

Rhapsody Health helps provider organizations—urban and rural—adopt and scale digital health solutions by removing integration and interoperability barriers. Our platform ensures seamless data exchange across EHRs, health information exchanges (HIEs), and emerging patient-facing applications.

- Support for HL7 v2, FHIR (including R4), X12, CDA, and custom formats to bridge legacy systems and new digital tools.
- Enable real-time and batch data exchange between systems, including FHIR-based APIs for patient and provider access.
- Integration of clinical and administrative workflows such as appointment scheduling, care planning, and decision support with APIs that normalize, route, and transform disparate data feeds.

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- Capabilities for NLP and unstructured data ingestion, facilitating the incorporation of faxed or scanned records into digital workflows.
- Rhapsody EMPI offers patient identity matching across source systems, essential for unified access and safe care transitions.

PR-2. Barriers to Workflow-Supporting Apps

Common obstacles include lack of EHR extensibility, proprietary data models, and difficulty integrating third-party tools into existing workflows. Rhapsody addresses this by acting as an abstraction and translation layer, enabling seamless bidirectional integration between EHRs and modern applications for documentation, billing, and quality reporting.

- Enable third-party tools to read/write into systems of record without disrupting core EHR configurations.
- Support FHIR Bulk Export and Group-based queries for reporting and analytics.
- Route clinical decision support triggers via CDS Hooks and facilitate app launch via SMART on FHIR.

Idea/Recommendation: CMS could standardize a modular integration pattern—"App-as-a-Service"—for documentation and billing applications, with published test beds for API interaction, easing vendor adoption. Also, extending the ONC Health IT Certification Program to include support for CDS Hooks and AI/ML transparency in clinical apps could incentivize innovation.

PR-3. Interoperability of All EHR Data Types

Rhapsody supports integration of structured and unstructured data, including scanned documents, PDF attachments, and free text. Our terminology services and NLP pipelines enable structured normalization and mapping to USCDI and SNOMED/LOINC standards.

- Ingest and normalize clinical documents from faxes, scanned images, and narrative notes.
- Assign standardized codes using built-in terminology services or third-party mappings.
- Make data interoperable for both human and machine consumption via FHIR.

Idea/Recommendation: CMS should expand USCDI to support metadata tags for unstructured formats and provide FHIR resource "wrappers" for scanned documents, increasing their interoperability. Investment in community-validated NLP libraries (open or certified) could also be subsidized to improve interpretability of legacy formats.

PR-4. Patient-Facing App Integration with Admin Workflows

Rhapsody enables secure, standards-based connections between third-party digital tools and scheduling, intake, and communication systems. This includes:

- Integrations with HL7 v2 ADT and scheduling messages.
- Mapping administrative functions to FHIR resources like `Appointment`, `Schedule`, and `QuestionnaireResponse`.

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Idea/Recommendation: CMS could mandate a core set of administrative FHIR resources for EHR vendors and promote a consumer scheduling API similar to Blue Button. Also, setting minimum functionality standards for appointment and intake workflows (like SMART forms) could increase parity across provider systems.

PR-5 to PR-8. FHIR API Support and Quality Reporting

Rhapsody supports all CMS-mandated FHIR APIs either natively or via proxy:

- Patient Access, Provider Access, Payer-to-Payer, Prior Authorization APIs
- Bulk FHIR with Group ID filtering
- SMART on FHIR with both standalone and EHR-launched modes
- CDS Hooks integration for third-party CDS tools, and supports

- Extract and transform data for clinical quality measurement reporting to CMS or internal dashboards.
- Integrate with registry and analytics platforms for population health and performance measurement.

PR-9 to PR-11. Digital Identity and TEFCA Participation

Rhapsody EMPI plays a pivotal role in identity assurance across systems by linking digital identities to clinical records through probabilistic and referential matching. We are aligned with IAL2 and AAL2 requirements**,** and support integration with Login.gov, ID.me, and other CSPs.

- Map federated digital credentials to enterprise identity systems.
- Enable TEFCA QHIN-level participation through data normalization and patient linking.

Idea/Recommendation: CMS could introduce a national patient digital identity broker service that works with both payers and providers. TEFCA implementation guidance should also include patient-facing use cases, not just B2B data exchange.

PR-12 to PR-14. Information Blocking and Disincentives

Rhapsody supports transparent and standards-based data exchange, and we've helped numerous organizations align with ONC's Information Blocking Rule by implementing APIs that expose the full USCDI dataset and enable EHI exports.

Idea/Recommendation: CMS might offer a compliance scorecard visible to patients and partners, incentivizing good data-sharing practices. Also, grants to help smaller providers implement certified FHIR endpoints via vendor-neutral hubs like Rhapsody could reduce the disparity in compliance capacity.

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Stakeholder Category – Payers

PA-1. Policy or Technical Limitations in TEFCA

We enable payers to connect to QHINs, normalize data across disparate sources, and ensure compliance with exchange protocols.

- Translate between FHIR, XCA, HL7 v2, and proprietary formats.
- Route payer data through TEFCA-compatible exchange frameworks.
- Implement patient identity matching and directory services for endpoint resolution.

Idea/Recommendation: CMS could expand TEFCA onboarding support for payers by funding technical intermediaries and creating payer-specific implementation toolkits. Additional guidance on TEFCA use cases for prior authorization and risk scoring would accelerate adoption.

PA-2. Accelerating Payer API Implementation

Rhapsody enables payers to deploy FHIR APIs rapidly and securely, even when underlying systems don't natively support them.

- Expose Patient Access, Provider Access, and Payer-to-Payer APIs.
- Support FHIR Bulk Data Export for population-level analytics.
- Bridge legacy claims systems and adjudication platforms to modern APIs.

Idea/Recommendation: CMS could incentivize adoption of payer APIs through a phased compliance timeline and offer implementation grants tied to specific API performance metrics (e.g., availability, latency, error rate).

PA-3. Digital Identity Credential Acceptance

Rhapsody supports payer integration with Login.gov, ID.me, and other CSPs that meet NIST IAL2/AAL2 standards. Our EMPI services link digital credentials to longitudinal patient records across multiple systems.

Idea/Recommendation: CMS might establish a federal trust framework for digital credentials used in both payer and provider settings, allowing beneficiaries to use one credential across all services. A pilot with MA plans could test usability and adoption.

PA-4. Value of a Nationwide FHIR Provider Directory

Rhapsody actively supports provider directory publishing and querying using FHIR `Endpoint`, `Practitioner`, `Organization`, and `Location` resources.

- Publish FHIR endpoints and link them to NPIs and payer contracts.
- Integrate provider directories into claims adjudication, referral, and authorization workflows.

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Idea/Recommendation: CMS could make the national directory queryable by network affiliation and contract type, enabling payers to streamline eligibility and claims workflows.

PA-5. Simplifying Quality Data Responsibilities

Rhapsody allows payers to extract, transform, and calculate quality measures based on clinical and claims data, minimizing the need for provider-side calculations.

- Ingest EHR data via FHIR or CCDs.
- Normalize and map to quality measure specifications.
- Support payer-side measure calculation for HEDIS and QPP.

PA-7. Encouraging Payers to Report Information Blocking

Rhapsody logs data request failures, delayed API responses, and malformed payloads—all indicators of potential information blocking.

Idea/Recommendation: CMS could establish a secure, automated reporting API where interoperability platforms like Rhapsody can push anonymized information blocking indicators (e.g., repeated 403 errors, empty responses to valid queries).

Stakeholder Category – Technology Vendors, Data Providers, and Networks

TD-1. Steps to Stimulate Developer Interest

Rhapsody supports hundreds of application developers and healthtech innovators by offering scalable, standards-based APIs, developer-friendly tools, and prebuilt connectors to payers, providers, and public health systems.

- Offer FHIR, HL7 v2, CDA, and X12 support out of the box.
- Provide a robust developer toolkit for building against interoperable endpoints.
- Enable app vendors to rapidly test and deploy integrations with provider systems and payer APIs.

Idea/Recommendation: CMS could fund "starter kits" for app developers that include mock patient data, sandbox environments, and test harnesses aligned to Blue Button and TEFCA. A certification accelerator program could also help startups prove CMS-readiness faster.

TD-2. Valuable CMS and Non-CMS Data Sources

Rhapsody aggregates and normalizes data from a broad spectrum of sources including:

- Medicare claims (Parts A, B, D)
- EHR data

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- Social determinants and community resource platforms
- Lab, pharmacy, and imaging systems, resulting in functionality like

- Link CMS data with non-CMS datasets in real time using patient identity resolution (EMPI).
- Normalize coding and terminology across sources for consistency and analytics.

Idea/Recommendation: CMS should enable plug-and-play data connectors (FHIR endpoints or bulk exports) for other federal agencies (e.g., VA, CDC, SSA), creating a more holistic longitudinal record. Enabling non-CMS sources to publish FHIR data aligned to USCDI would reduce mapping friction for developers.

TD-3. Digital Identity Implementation

Rhapsody integrates with digital identity providers and supports Identity Assurance Level 2 (IAL2) requirements for TEFCA and beyond. Our EMPI platform links credentials to clinical records across organizations.

- Integrate with Login.gov, ID.me, CLEAR, and enterprise IDPs.
- Ensure secure identity federation and resolution for patient and provider access.

Idea/Recommendation: CMS could publish a national digital identity framework that mandates support for NIST 800-63-3 standards while offering grants for community-based credential enrollment. Broader access to identity services could be enabled through mobile driver's licenses and pharmacy-based identity proofing.

TD-4. Promoting Open API Use

Rhapsody encourages the use of open, standards-based APIs and offers full lifecycle support to organizations transitioning from proprietary integrations.

- Translate proprietary EHR interfaces to FHIR using adapters and protocol transformers.
- Ensure API security, throttling, and audit logging as part of an enterprise-grade middleware platform.

TD-5. Nationwide FHIR Provider Directory

Rhapsody already supports publishing and consuming FHIR `Endpoint`, `PractitionerRole`, and `Organization` resources to enable provider directories and routing logic.

- Act as an intermediary to manage directory publication.
- Link digital identity credentials with endpoint metadata.

Idea/Recommendation: CMS could host the central directory but allow trusted integration vendors to update it using authenticated API calls. A self-service UI with bulk upload and validation tools would improve adoption.

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TD-6. TEFCA's Unique Interoperability Role

Rhapsody is working with clients on TEFCA-readiness and understands its unique value in enabling nationwide, policy-governed, trust-based exchange, especially where EHR vendor networks remain fragmented.

Idea/Recommendation: CMS and ONC should ensure that TEFCA supports FHIR APIs natively—not just document-based exchange (e.g., XCA). Also, grant funding for onboarding smaller hospitals and FQHCs via intermediary QHINs would improve equitable participation.

TD-7. USCDI Effectiveness and Limitations

Rhapsody supports USCDI v3 and maps legacy or non-standardized data into USCDI fields via terminology services.

Idea/Recommendation: USCDI expansion should be modular—allowing for "USCDI +" profiles that are domain-specific (e.g., behavioral health, pediatrics). CMS could also standardize "confidence scores" for data quality and completeness across USCDI elements.

TD-8 & TD-9. Effective Certification Criteria and API Emphasis

Rhapsody supports customers in meeting ONC certification by offering FHIR-first connectivity, bulk data export, and structured/unstructured data transformation.

- Support EHI export (single and population level)
- Translate EHR-native formats into ONC-recognized APIs and terminologies.

Idea/Recommendation: ONC certification should prioritize "API completeness" — including document retrieval, patient-written data, and free-text narratives. Certification test cases should reflect real-world interoperability demands, not just minimum specs.

TD-10 & TD-11. Full EHR Data Access and EHI Export

Rhapsody enables access to all data types across clinical systems, including:

- Narrative notes
- Scanned and faxed documents
- Structured fields not currently mapped to USCDI

Idea/Recommendation: CMS could establish an "EHI Completeness Index" based on actual export contents and coverage. Requiring a consistent JSON export template (aligned to FHIR) for EHI would drastically simplify downstream consumption.

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TD-12. Endorsement of Non-CMS Data Sources

Rhapsody believes that high-value data sources like social services, imaging networks, pharmacy benefit managers (PBMs), and behavioral health EHRs should be considered for inclusion.

Idea/Recommendation: CMS could create an "Interoperability Partner Network" that includes endorsed non-CMS data contributors. Participants could be scored based on data quality, frequency, and alignment to federal standards.

TD-19. Price Transparency and Impactful Workflows

Rhapsody enables transmission of cost data using FHIR `ChargeItem`, `ExplanationOfBenefit`, and `Coverage` resources.

Idea/Recommendation: CMS could promote real-time pricing APIs that include contextual variables (e.g., site of care, coverage tier). Linking price transparency with scheduling and quality ratings could empower true patient choice.

Stakeholder Category – Value-Based Care Organizations

VB-1. Incentives for APMs to Adopt Digital Health Tools

Rhapsody enables APM participants, including ACOs and MSSP organizations, to integrate digital health tools into care workflows by offering a robust, vendor-neutral platform that connects EHRs, HIEs, claims data, and patient apps.

- Integrate claims, clinical, and SDOH data to inform care management.
- Enable digital tools that support chronic disease tracking, patient-reported outcomes, and shared decision-making.
- Power patient engagement applications with real-time data feeds via FHIR APIs.

Idea/Recommendation: CMS could tie incentive payments to measured digital tool adoption (e.g., % of attributed patients using CMS-validated tools) and offer implementation support grants for small/rural ACOs.

VB-2. Integration of Key Technologies in APM Requirements

Rhapsody supports:

- AI-driven risk stratification through normalized data pipelines.
- Population health analytics by aggregating data across payers and providers.
- Remote device data integration for real-time monitoring.

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Idea/Recommendation: CMS could establish a "Digital Core Set" of capabilities that APMs can adopt (e.g., care coordination platform, real-time risk dashboards, patient messaging tools) and recognize their use in performance scoring.

VB-3. Essential IT Capabilities for VBC

Rhapsody powers many of the functions listed, including:

- Attribution and patient ID matching (EMPI)
- Quality performance data aggregation and normalization
- Event notification integration via HL7 and FHIR
- Remote monitoring ingestion (via device APIs and intermediaries)

Idea/Recommendation: CMS could create a modular tech readiness framework for APMs, allowing organizations to select verified tools for data extraction, ID resolution, and patient engagement based on their specific maturity level.

VB-4. Essential Data Types for VBC

Rhapsody enables access to and harmonization of:

- Claims and utilization data
- EHR-derived clinical metrics
- Lab results and diagnostics
- Behavioral and SDOH data
- Patient-reported outcomes and device telemetry

Idea/Recommendation: CMS could establish a "Value-Based Data Index" model that quantifies completeness, timeliness, and accessibility of these datasets across APMs._

VB-7. Simplifying Technology Requirements for APMs

Rhapsody helps organizations abstract complexity by enabling vendor-agnostic data exchange and reducing reliance on bespoke EHR customization.

Idea/Recommendation: CMS could consolidate reporting and interoperability mandates under one flexible framework, allowing data submission via either registry, EHR, or validated third-party intermediaries.

VB-8. Optimizing CEHRT via Other HHS Policies

Rhapsody supports transmission of electronic patient event notifications (ADT messages and FHIR subscriptions) to downstream care teams and community providers.

Idea/Recommendation: CMS could extend Conditions of Participation to include event notification for APM entities, particularly for transitions of care and high-risk readmissions.

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VB-9. Unique Requirements for APM vs. Non-APM Organizations

APMs often require more dynamic patient tracking, flexible attribution updates, and near real-time insights. Rhapsody enables this through high-frequency data refresh and cross-entity normalization.

Idea/Recommendation: CMS could stratify requirements by provider type and designate APM-specific pathways for meeting interoperability, allowing for modular certification or performance exceptions based on organization size and risk exposure.

VB-10. Flexible CEHRT Criteria for Advanced APMs

Rhapsody enables clinical organizations to meet CEHRT objectives through external API services and reporting pipelines without having to overhaul existing systems.

Idea/Recommendation: CMS could formalize a "Bring Your Own Tech Stack" approach for Advanced APMs, with validation around functionality and outcomes rather than prescriptive tools.

VB-11. Interoperability Challenges in VBC

Challenges include:

- Lack of consistent patient identifiers
- Data fragmentation across multiple care settings
- Latency in data availability for risk and performance metrics

How Rhapsody helps:

- EMPI resolves cross-system ID issues.
- Aggregation from payer and provider feeds ensures completeness.
- Real-time APIs reduce lag for care interventions.

VB-12. Technology Standardization vs. Flexibility

Rhapsody provides standard-based APIs but allows for custom routing, filtering, and transformation to adapt to each organization's unique workflows.

Idea/Recommendation: CMS could endorse a "Core + Configurable" standards model: a base standard set required across programs, with optional extensions tailored to clinical specialty or community needs.

VB-13. Standards to Support VBC and Reduce Burden

Rhapsody reduces burden by combining data from multiple sources into unified formats for submission, dashboards, or partner systems. Our platform also enables automatic measure calculations where possible.

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Idea/Recommendation: CMS could co-develop shared measure libraries (e.g., as FHIR `Measure` and `MeasureReport` resources) that are computable and versioned across all APMs.

VB-14. Digital Identity's Role in VBC

Accurate digital identity is foundational for attribution, consent management, and secure communication. Rhapsody EMPI links digital IDs to patient records across entities.

Idea/Recommendation: CMS could explore a federated identity model for attributed patients, enabling trusted access across all participants in an ACO or other value-based network.

VB-15. Nationwide Provider Directory of FHIR Endpoints

Rhapsody supports publishing and querying provider directories including endpoint metadata, NPI crosswalks, and credential links.

Idea/Recommendation: A centralized CMS-managed directory with real-time sync APIs would significantly accelerate point-of-care interoperability and attribution accuracy. Key data elements should include `Endpoint`, `NPI`, `PractitionerRole`, credential assurance level, and accepted insurance networks.

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